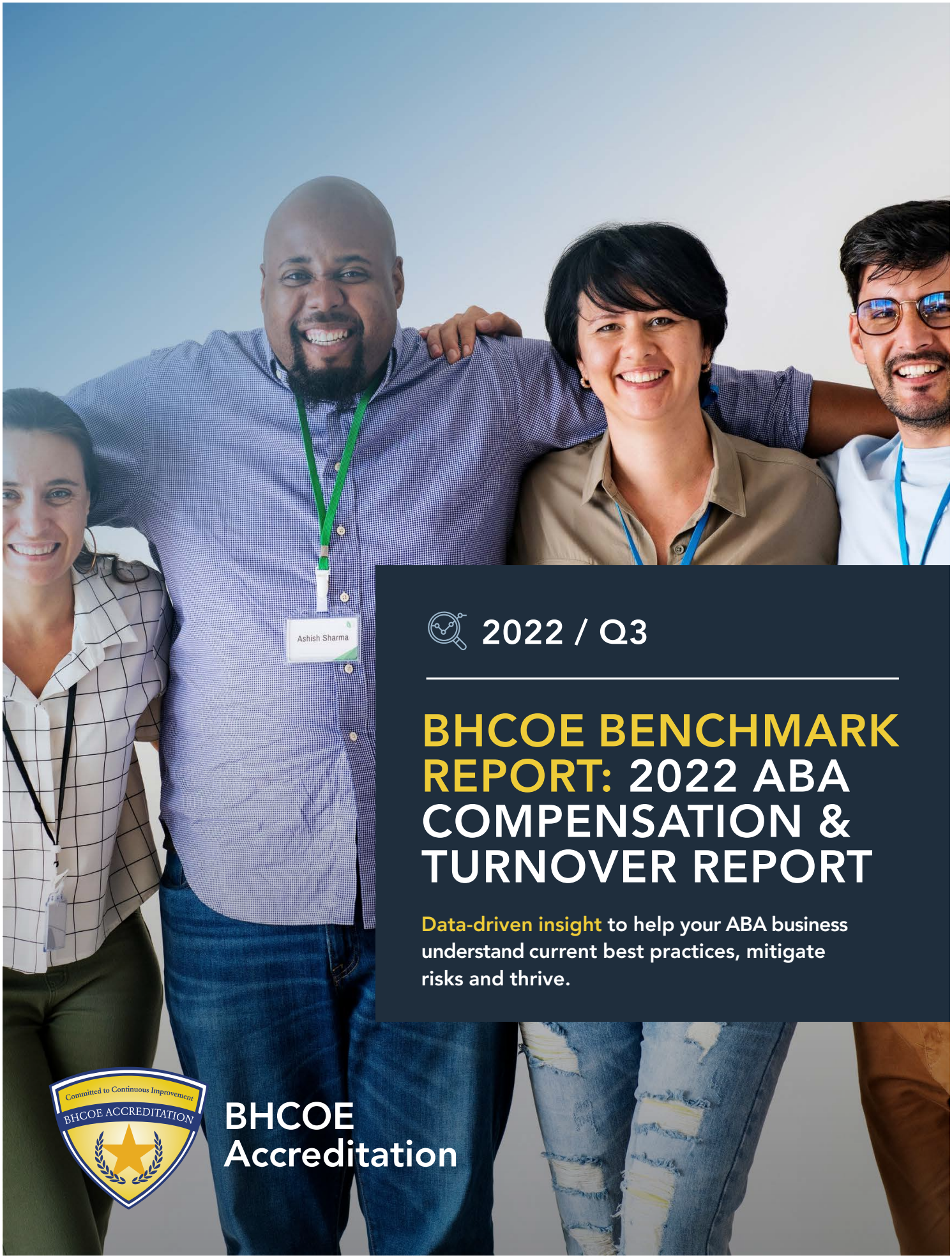


2022 / Q3

BHCOE BENCHMARK REPORT: 2022 ABA COMPENSATION & TURNOVER REPORT

Data-driven insight to help your ABA business understand current best practices, mitigate risks and thrive.

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EXECUTIVE SUMMARY

Compensation and staff turnover are two major pain points for every provider. Although they are not always strongly related, both have a tremendous impact on the financial health and sustainability of ABA organizations. The purpose of this report is to describe industry trends in hourly pay rate, annual salaries, and annualized turnover percentage for ABA organizations who completed the BHCOE evaluation process. Like past reports, we describe the average pay rate, annual salaries, and turnover percentages for direct staff, supervisors, and for clinical directors. We also include data around a new metric—staff tenure—to highlight an alternative way to consider employment for roles where employees are expected to leave the position at some point during their career (e.g., behavior technicians).

The data for this report were collected during the BHCOE evaluation process. The previous BHCOE annual compensation and turnover report[1] provided information up through 2020 with the average hourly pay rates and percentage turnover for direct staff and for supervisory staff. For this report, we aggregated data from organizations who completed the BHCOE evaluation process during 2021 and up through July of 2022. A total of 138 organizations provided data on hourly pay rates and 98 organizations provided data on turnover percentages.

Direct care staff average pay rates increased from $18.99 in 2020 to $20.42 in 2021 (increase of 7.53%); and, supervisor average pay rates increased from $76,752 annually in 2020 to $94,640 annually in 2021 (increase of 23.31%). During the same period in the United States, the average inflation rate was 7%[2].

From 2020 to 2021, turnover percentages increased from 59% to 65% for direct care staff; decreased from 25% to 19% for supervisory staff; and decreased from 22% to 5% for clinical director-level staff. Organizations stopped reporting COVID-related furloughs and terminations disappeared in 2021, suggesting either that COVID-19's impact on turnover was isolated to the first year of the pandemic, or the lingering effects of COVID-19 on turnover is attributed to alternative causes.

[1] https://www.bhcoe.org/project/2021-aba-compensation-and-turnover-report/

[2] https://www.bls.gov/opub/ted/2022/consumer-price-index-2021-in-review.htm

SECTION I: COMPENSATION

We recognize that monetary compensation is one part of the overall compensation package an employee receives (e.g., continuing education, healthcare benefits, savings). Researchers have found that non-monetary compensation (e.g., career development, job satisfaction, managerial support) can more significantly influence turnover than monetary compensation for some employees[3,4]. Further, individual employees are likely to differ in the type of job environment and experiences that lead them to be happier with their job. Nevertheless, knowledge of employee pay in a market is important to successfully recruit and retain employees. Thus, we provide the information below to help the field understand what monetary compensation looks like in this sample of ABA providers.

Limitations & Data Handling

The data below did not control for factors that are likely to influence the amount of monetary compensation that an employee receives. For example, compensation is likely influenced by years of experience, market demand for ABA services, and negotiated reimbursement rates. None of these were controlled for in the descriptions below. In addition, the sample is comprised solely of organizations that completed the BHCOE evaluation process. These organizations often invest in systems to improve employee satisfaction. Thus, the data in this report are likely biased, though the extent to which these data differ from non-accredited organizations is unknown. However, these variables are likely to be randomly distributed across all organizations and regions and, thus, may influence the pay rate for any one individual employee, but not for the dataset as a whole.

All wage data were provided for either part-time or full-time employees. If data were provided for independent contractors, they were excluded. Data were provided in either an hourly or salary format. Wage data for all employees were converted to an hourly rate based on a 40-hour work week and 52-week per year estimate.

It has also been suggested that pay rates and turnover might differ between employees working primarily in clinic versus home settings. Business operations and resulting flexibility in employee compensation are likely impacted by constraints determined by the primary work setting of their employees. For example, an organization that operates primarily out of a clinic setting may have constraints stemming from the overhead of operating out of a physical location. In contrast, an organization operating primarily out of home settings will likely have less overhead but will have considerations around employee mileage reimbursement and the logistics of drive times. The influence of these kinds of economics factors may influence what organizations can offer their employees.

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[3] Cymbal, D. J., Litvak, S., Wilder, D. A., & Burns, G. N. (2022). An examination of variables that predict turnover, staff and caregiver satisfaction in behavior-analytic organizations. *Journal of Organizational Behavior Management, 42*(1), 36–55.

[4] Kazemi, E., Shapiro, M., & Kavner, A. (2015). Predictors of intention to turnover in behavior technicians working with individuals with autism spectrum disorder. *Research in Autism Spectrum Disorders, 17*, 106–115.

Data are best understood in context. There are at least two variables you should consider when interpreting the data below: inflation and cost of living (CoL). **Inflation** can vary considerably by region, state, locale, and—with an increasingly connected global economy—even by good (e.g., change in costs of bread vs. oil vs. housing). It is nearly impossible to control for the impact of such nuanced inflation for any one employee in any one location. Additionally, inflation related to gas prices is likely to impact direct care staff commuting 2-3 hours per day in California traffic differently than a Supervisor working in a clinic 3 miles from their home in Iowa. Nevertheless, we did include information on Cost of Living (CoL) by state. CoL estimates were obtained from the Council for Community and Economic Research[5] and states were subsequently categorized into quintiles (e.g., lowest 20% CoL group, 20%-40% CoL group, 40%-60%, 60%-80%, 80%-100%, with the top group including 11 territories to include the District of Columbia). CoL estimates consider current prices for goods and services in various categories such as housing, food, utilities, transportation, healthcare, and miscellaneous services (e.g., dry cleaning, haircut). This allows us to describe monetary compensation across levels of CoL so you can make comparisons among approximately similar economic states.

Summary of Results

Since the 2021 report, 15 states changed CoL quintiles. Seven states moved to a higher CoL group: Arizona, Arkansas, Idaho, Michigan, New Mexico, Vermont, and Wyoming. Eight states moved to a lower CoL group: Alabama, Indiana, Iowa, Minnesota, Montana, New Jersey, South Carolina, and West Virginia. The top 10 most expensive states were slightly different than the last report with Rhode Island moving into the top 10 (previously number 11) and New Jersey moving out of the top 10.

The average direct care staff hourly pay increased for four groups and decreased for one group: Group 1 increased by 0.83%; Group 2 decreased by 0.7%; Group 3 increased by 13.24%; Group 4 increased by 22.31%; and Group 5 increased by 6.87%. The average supervisor pay increased for two groups and decreased for three groups: Group 1 decreased by 9.55%; Group 2 decreased by 12.32%; Group 3 decreased by 3.09%; Group 4 increased by 26.26%; and Group 5 increased by 32.28%.

In this report we added wage data for clinical directors, which we did not include in the 2021 report. The average wages for clinical directors were much less variable than those for direct care staff and supervisors. Among clinical directors, group 1 had the lowest average annual estimated reported salary ($83,012 per year) and Group 4 had the highest average annual estimated reported salary ($121,264 per year).

[5] https://www.c2er.org/

Average Hourly Rates by Cost-of-Living Group: Direct Care Staff

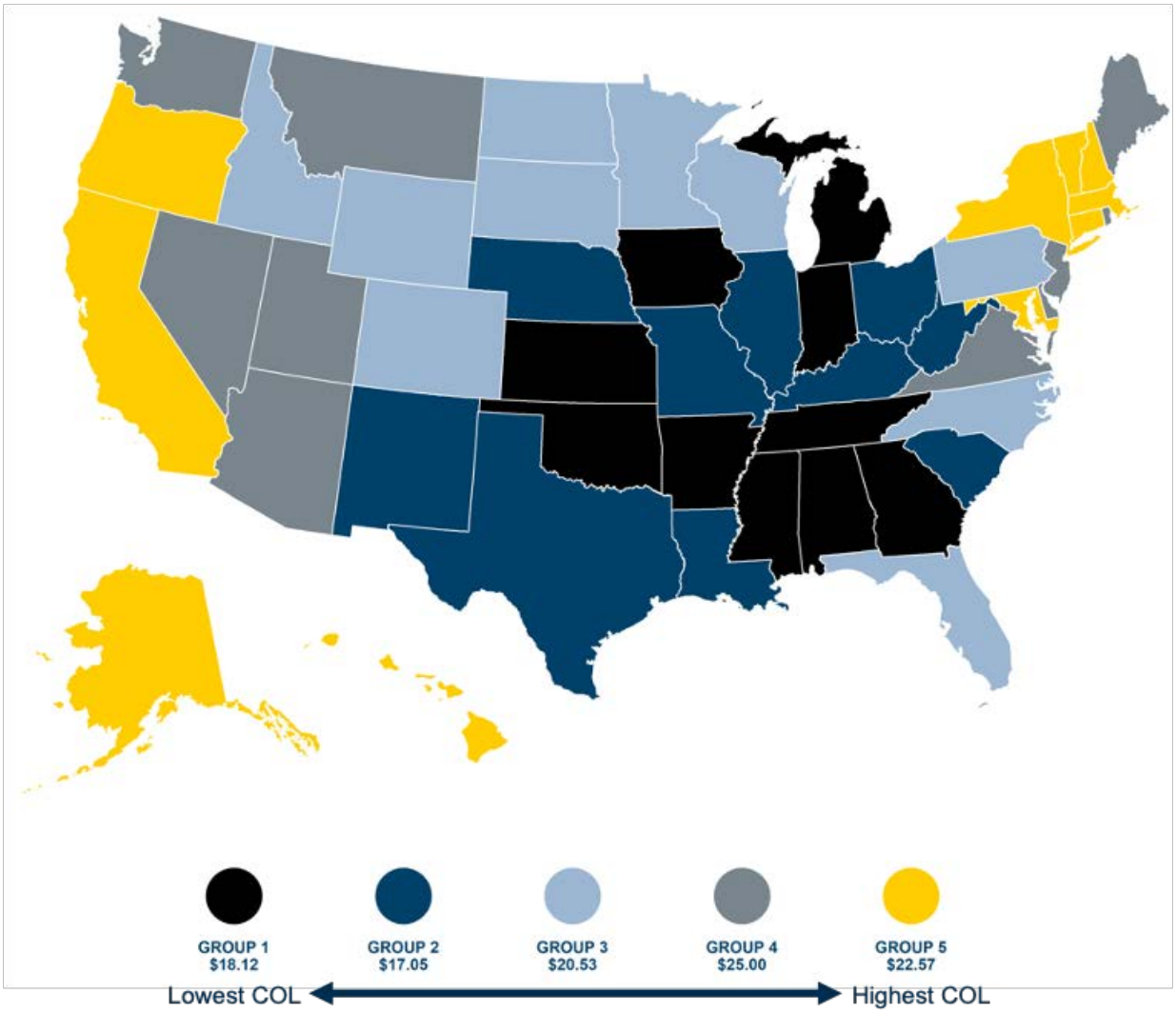


Figure 1. States were sorted by cost-of-living index and then divided into five cost of living indexes (Groups 1 – 5; denoted by the different colors in the plot above). Group 1 includes the 10 least expensive states; Group 5 includes the 10 most expensive states and Washington DC. Average hourly rates were calculated within each cost-of-living group after removing values more than three standard deviations away from the average of each group. Hourly rates were self-reported by organizations during the BHCOE accreditation process. Hourly rates reported for direct care staff did not include alternative rates used for drive-time or non-billable work. More in-depth details about each cost-of-living group can be found on the next page.

Cost of Living Groups Summary: Direct Care Staff

GROUP	STATE	COST OF LIVING RANK	COST OF LIVING INDEX SCORE	AVERAGE HOURLY RATE	MEDIAN HOURLY RATE	MINIMUM AVERAGE ORG RATE	MAXIMUM AVERAGE ORG RATE	STAFF IN THE SAMPLE	ORGS IN THE SAMPLE
1	Mississippi	1	83.1	$18.12	$18.00	$14.66	$22.82	3,578	30
	Oklahoma	2	84.8						
	Kansas	3	85.4						
	Alabama	4	87.5						
	Iowa	5	88.1						
	Georgia	6	88.2						
	Indiana	7	89.2						
	Tennessee	8	89.4						
	Arkansas	9	89.9						
	Michigan	10	89.9						
2	West Virginia	11	90.5	$17.05	$17.00	$14.22	$21.78	1,498	21
	Missouri	12	90.5						
	Ohio	13	90.8						
	Illinois	14	91.7						
	Nebraska	15	91.8						
	Texas	16	92.1						
	Louisiana	17	93.3						
	Kentucky	18	93.6						
	New Mexico	19	93.9						
	South Carolina	20	93.9						
3	Wyoming	21	93.9	$20.53	$20.00	$16.35	$25.47	3,826	39
	North Carolina	22	95.0						
	South Dakota	23	95.2						
	Wisconsin	24	95.5						
	North Dakota	25	96.8						
	Minnesota	26	97.2						
	Pennsylvania	27	97.5						
	Idaho	28	99.4						
	Colorado	29	102.4						
	Florida	30	102.6						
4	Virginia	31	102.7	$25.00	$25.00	$18.80	$34.94	1,240	18
	Nevada	32	103.1						
	Utah	33	104.2						
	Montana	34	104.4						
	Delaware	35	105.8						
	Arizona	36	106.7						
	Rhode Island	37	112.8						
	New Jersey	38	112.9						
	Washington	39	113.9						
	Maine	40	115.1						
5	Vermont	41	116.9	$22.57	$22.00	$17.92	$27.07	4,090	34
	New Hampshire	42	117.4						
	Connecticut	43	118.9						
	Oregon	44	125.2						
	Maryland	45	126.4						
	Alaska	46	127.3						
	Massachusetts	47	130.2						
	California	48	139.7						
	New York	49	152.1						
	District of Columbia	50	158.8						
	Hawaii	51	192.7						

Distribution of Hourly Rates: Direct Care Staff

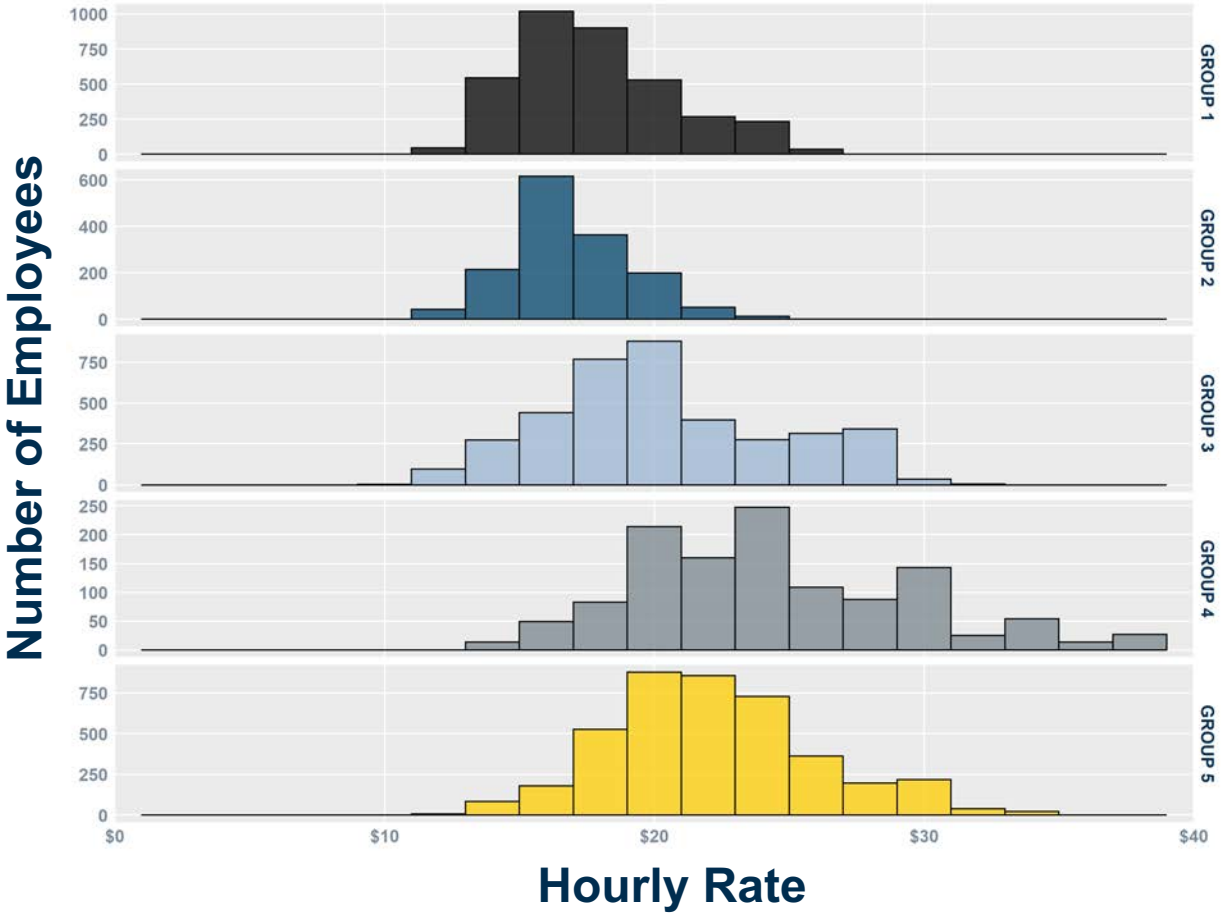


Figure 2. Distributions of hourly wage data for direct care staff for each of the five cost-of-living groups. Data are from direct care staff working at organizations that underwent a BHCOE evaluation in 2021 and through July of 2022. Hourly rates were self-reported by organizations during that process. Hourly rates reported for direct care staff did not include alternative rates used for drive-time or non-billable work.

Direct Staff Hourly Rates by Employment Status

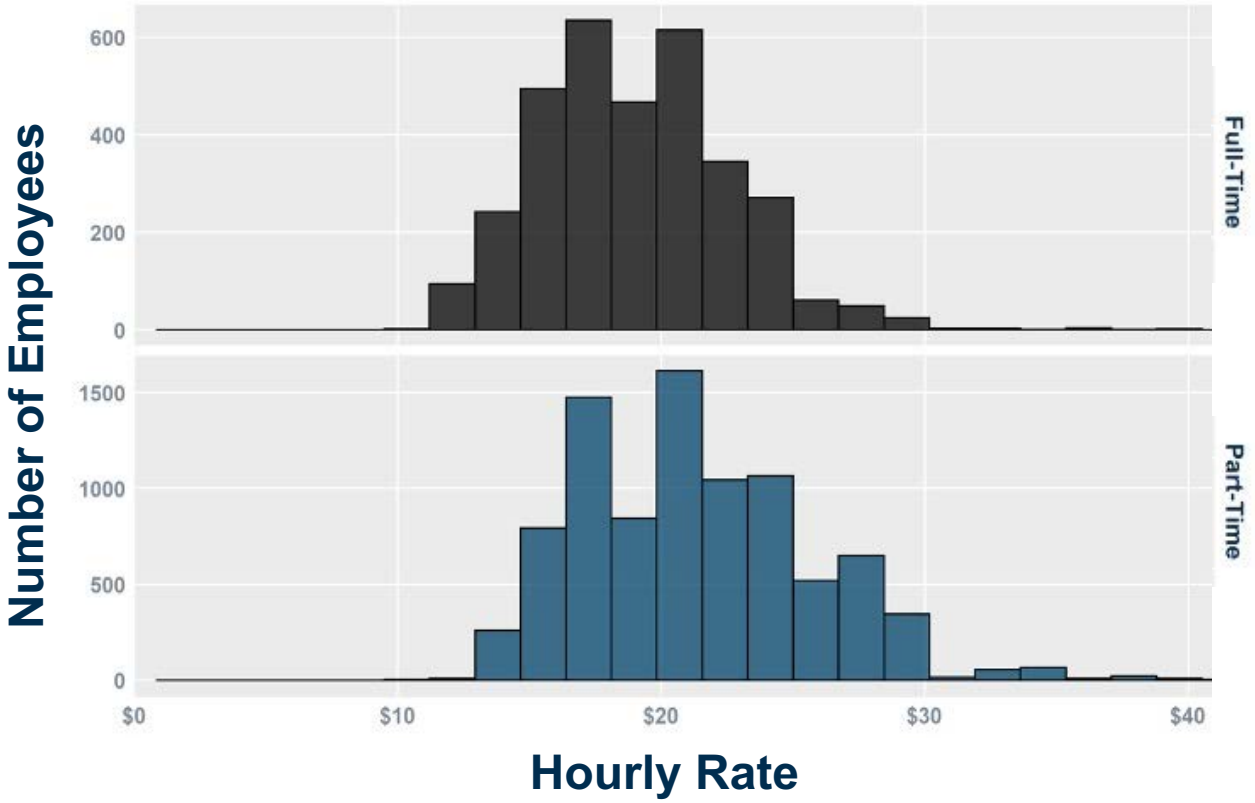


Figure 3. Distributions of hourly wage data for direct care staff by employment status. Data are from direct care staff working at organizations that underwent a BHCOE evaluation in 2021 and through July of 2022. Hourly rates were self-reported by organizations during that process. Part/full-time status was not reported for some individuals. Hourly rates reported for direct care staff did not include alternative rates used for drive-time or non-billable work.

Average Hourly Rates by Cost-of-Living Group: Supervisory Staff

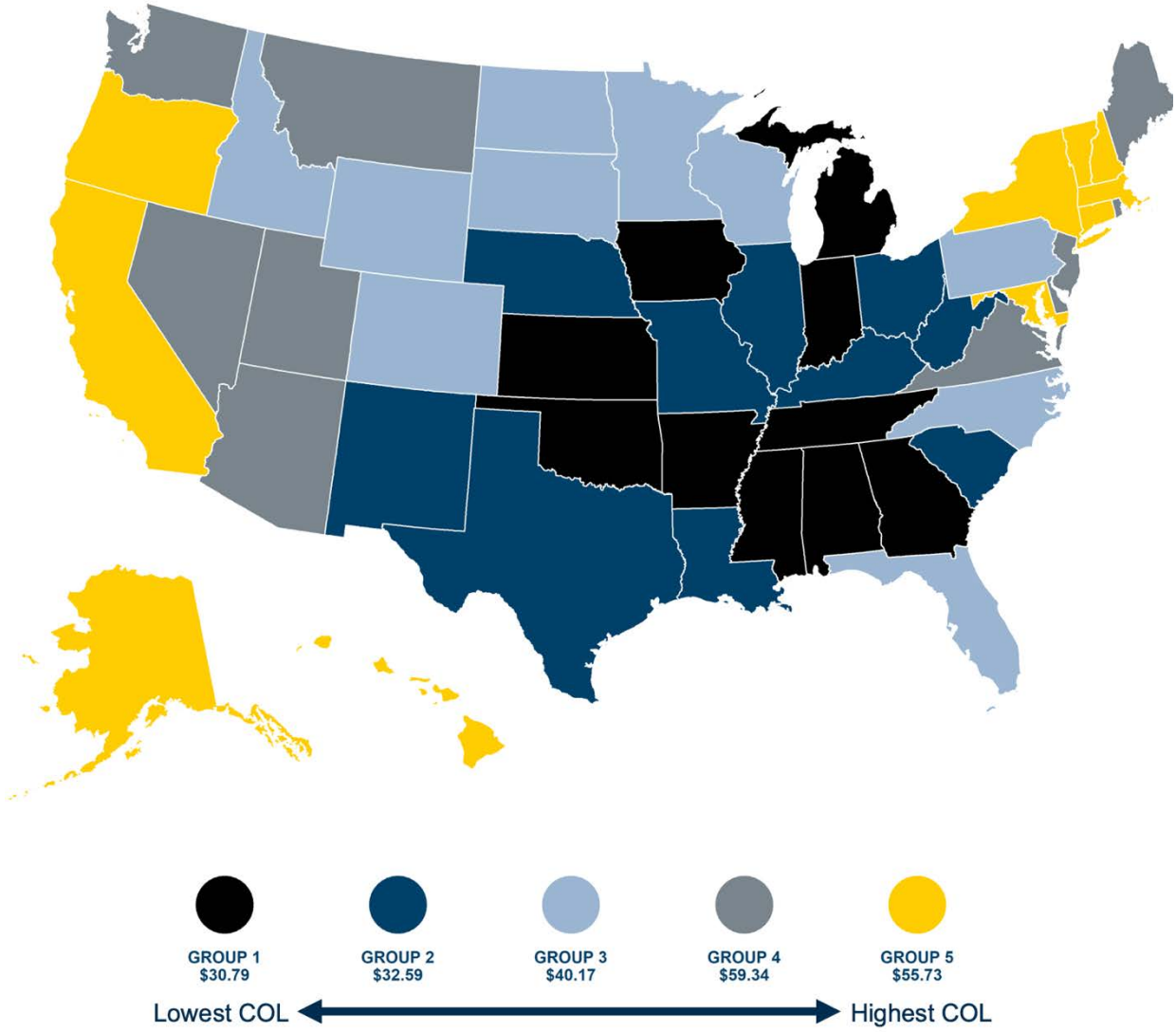


Figure 4. States were sorted by cost-of-living index and then divided into five cost of living indexes (Groups 1 – 5; denoted by the different colors in the plot above). Group 1 includes the 10 least expensive state; Group 5 includes the 10 most expensive states and Washington DC. Average hourly rates were calculated within each cost-of-living group after removing values more than three standard deviations away from the average of each group. Hourly rates were self-reported by organizations during the BHCOE accreditation process. Hourly rates reported for supervisory staff did not include alternative rates used for drive-time or non-billable work. More in-depth details about each cost-of-living group can be found on the next page.

Cost of Living Groups Summary: Supervisory Staff

GROUP	STATE	COST OF LIVING RANK	COST OF LIVING INDEX SCORE	AVERAGE HOURLY RATE	MEDIAN HOURLY RATE	MINIMUM AVERAGE ORG RATE	MAXIMUM AVERAGE ORG RATE	STAFF IN THE SAMPLE	ORGS IN THE SAMPLE
1	Mississippi	1	83.1	$30.79	$29.90	$29.31	$44.80	522	26
	Oklahoma	2	84.8						
	Kansas	3	85.4						
	Alabama	4	87.5						
	Iowa	5	88.1						
	Georgia	6	88.2						
	Indiana	7	89.2						
	Tennessee	8	89.4						
	Arkansas	9	89.9						
	Michigan	10	89.9						
2	West Virginia	11	90.5	$32.59	$33.65	$26.79	$39.99	330	20
	Missouri	12	90.5						
	Ohio	13	90.8						
	Illinois	14	91.7						
	Nebraska	15	91.8						
	Texas	16	92.1						
	Louisiana	17	93.3						
	Kentucky	18	93.6						
	New Mexico	19	93.9						
	South Carolina	20	93.9						
3	Wyoming	21	93.9	$40.17	$36.06	$29.75	$51.04	659	39
	North Carolina	22	95.0						
	South Dakota	23	95.2						
	Wisconsin	24	95.5						
	North Dakota	25	96.8						
	Minnesota	26	97.2						
	Pennsylvania	27	97.5						
	Idaho	28	99.4						
	Colorado	29	102.4						
	Florida	30	102.6						
4	Virginia	31	102.7	$59.34	$55.29	$28.15	$65.60	425	19
	Nevada	32	103.1						
	Utah	33	104.2						
	Montana	34	104.4						
	Delaware	35	105.8						
	Arizona	36	106.7						
	Rhode Island	37	112.8						
	New Jersey	38	112.9						
	Washington	39	113.9						
	Maine	40	115.1						
5	Vermont	41	116.9	$55.73	$43.27	$35.29	$60.42	947	34
	New Hampshire	42	117.4						
	Connecticut	43	118.9						
	Oregon	44	125.2						
	Maryland	45	126.4						
	Alaska	46	127.3						
	Massachusetts	47	130.2						
	California	48	139.7						
	New York	49	152.1						
	District of Columbia	50	158.8						
	Hawaii	51	192.7						

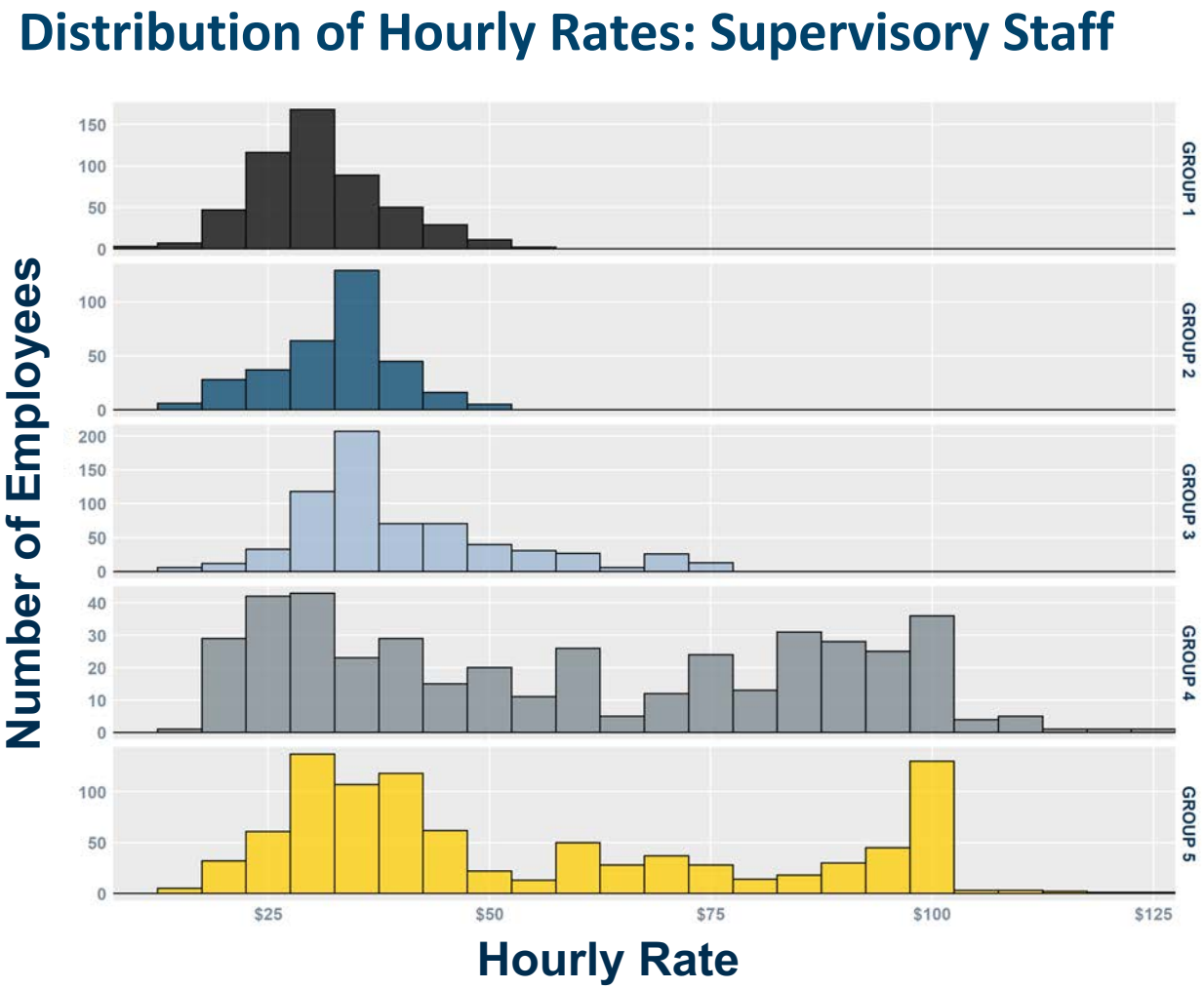


Figure 5. Distributions of hourly wage data for supervisory staff for each of the five cost-of-living groups. Data are from supervisory staff working at organizations that underwent a BHCOE evaluation in 2021 and through July of 2022. Hourly rates were self-reported by organizations during that process. Hourly rates reported for supervisory staff did not include alternative rates used for drive-time or non-billable work.

Supervisory Staff Hourly Rates by Employment Status

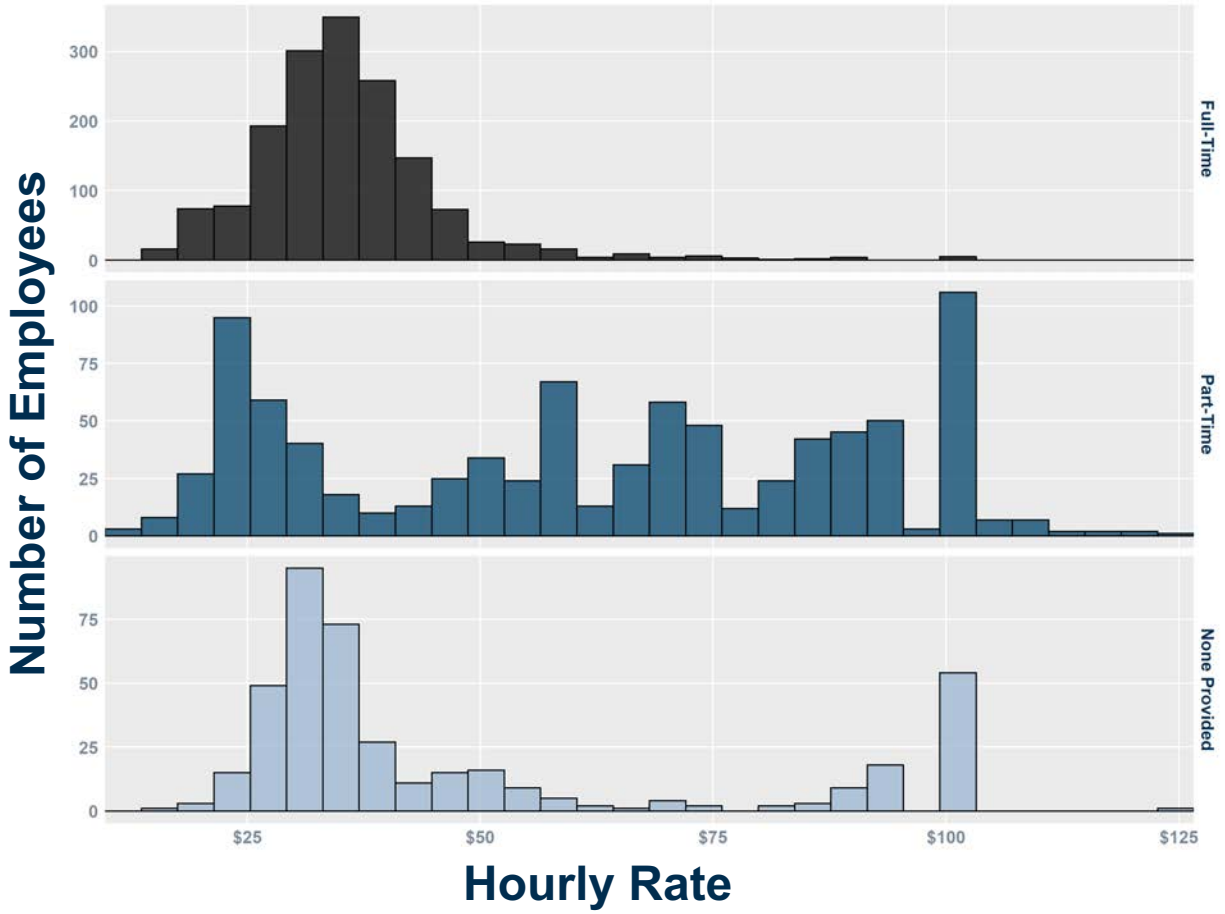


Figure 6. Distributions of hourly rate data for supervisory staff by employment status. Data are from supervisors working at organizations that underwent a BHCOE evaluation in 2021 and through July of 2022. Hourly rates were self-reported by organizations during that process. Hourly rates reported for supervisory staff did not include alternative rates used for drive-time or non-billable work.

Average Salary by Cost-of-Living Group: Clinical Directors

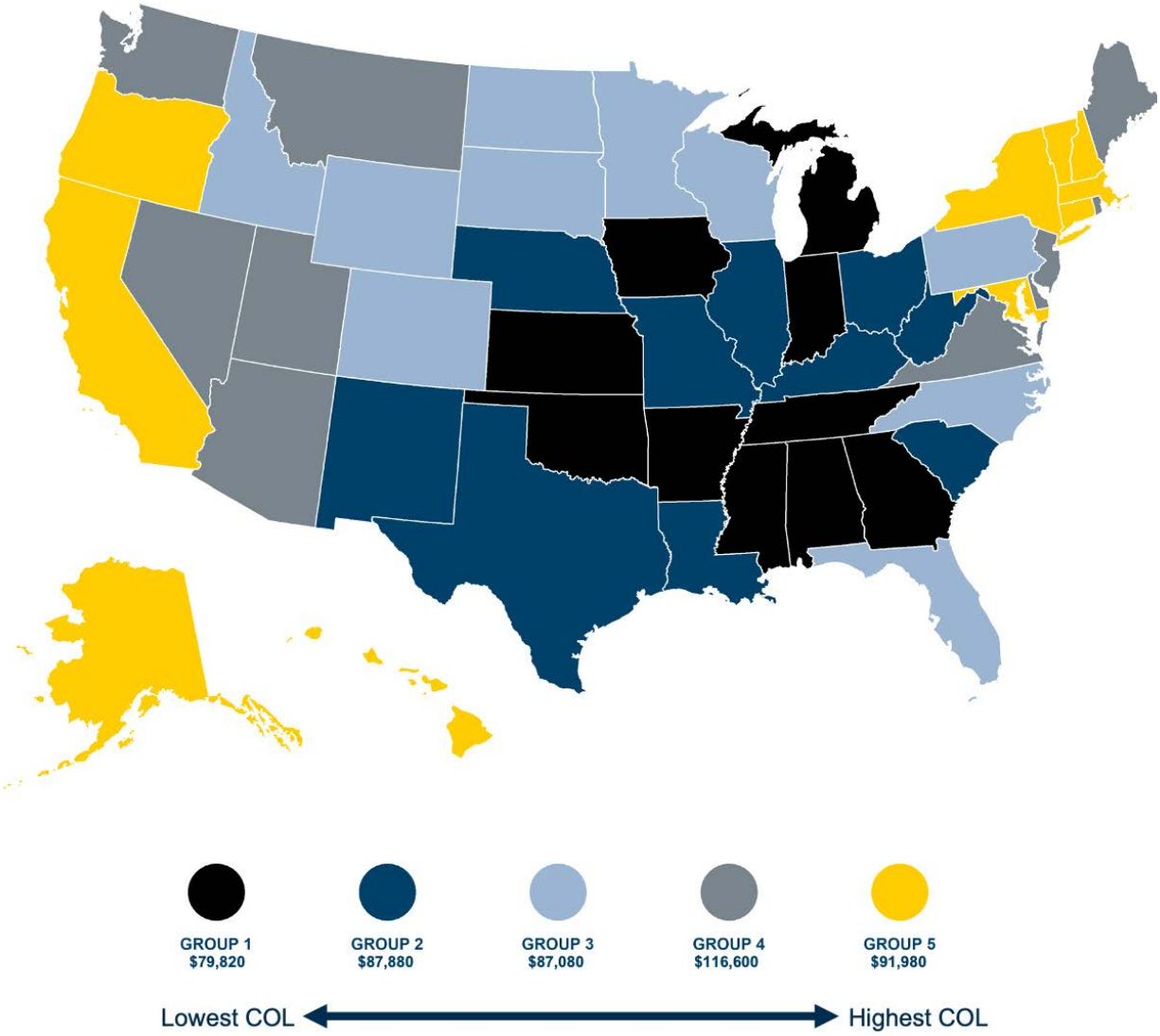


Figure 7. States were sorted by cost-of-living index and then divided into five cost of living indexes (Groups 1 – 5; denoted by the different colors in the plot above). Group 1 includes the 10 least expensive states; Group 5 includes the 10 most expensive states and Washington DC. Average salary was calculated within each cost-of-living group after removing values more than three standard deviations away from the average of each group. Salaries were self-reported by organizations during the BHCOE accreditation process. More in-depth details about each cost-of-living group can be found on the next page.

Cost of Living Groups Summary: Clinical Directors

GROUP	STATE	COST OF LIVING RANK	COST OF LIVING INDEX SCORE	AVERAGE HOURLY RATE	MEDIAN HOURLY RATE	MINIMUM AVERAGE ORG RATE	MAXIMUM AVERAGE ORG RATE	STAFF IN THE SAMPLE	ORGS IN THE SAMPLE
1	Mississippi	1	83.1	$79,820	$81,740	$77,180	$82,840	34	11
	Oklahoma	2	84.8						
	Kansas	3	85.4						
	Alabama	4	87.5						
	Iowa	5	88.1						
	Georgia	6	88.2						
	Indiana	7	89.2						
	Tennessee	8	89.4						
	Arkansas	9	89.9						
	Michigan	10	89.9						
2	West Virginia	11	90.5	$87,880	$85,560	$73,940	$88,640	28	10
	Missouri	12	90.5						
	Ohio	13	90.8						
	Illinois	14	91.7						
	Nebraska	15	91.8						
	Texas	16	92.1						
	Louisiana	17	93.3						
	Kentucky	18	93.6						
	New Mexico	19	93.9						
	South Carolina	20	93.9						
3	Wyoming	21	93.9	$87,080	$86,540	$85,400	$92,860	65	18
	North Carolina	22	95.0						
	South Dakota	23	95.2						
	Wisconsin	24	95.5						
	North Dakota	25	96.8						
	Minnesota	26	97.2						
	Pennsylvania	27	97.5						
	Idaho	28	99.4						
	Colorado	29	102.4						
	Florida	30	102.6						
4	Virginia	31	102.7	$116,600	$124,720	$96,460	$117,780	12	7
	Nevada	32	103.1						
	Utah	33	104.2						
	Montana	34	104.4						
	Delaware	35	105.8						
	Arizona	36	106.7						
	Rhode Island	37	112.8						
	New Jersey	38	112.9						
	Washington	39	113.9						
	Maine	40	115.1						
5	Vermont	41	116.9	$91,980	$81,740	$71,120	$109,900	50	10
	New Hampshire	42	117.4						
	Connecticut	43	118.9						
	Oregon	44	125.2						
	Maryland	45	126.4						
	Alaska	46	127.3						
	Massachusetts	47	130.2						
	California	48	139.7						
	New York	49	152.1						
	District of Columbia	50	158.8						
	Hawaii	51	192.7						

Distribution of Salaries: Clinical Directors

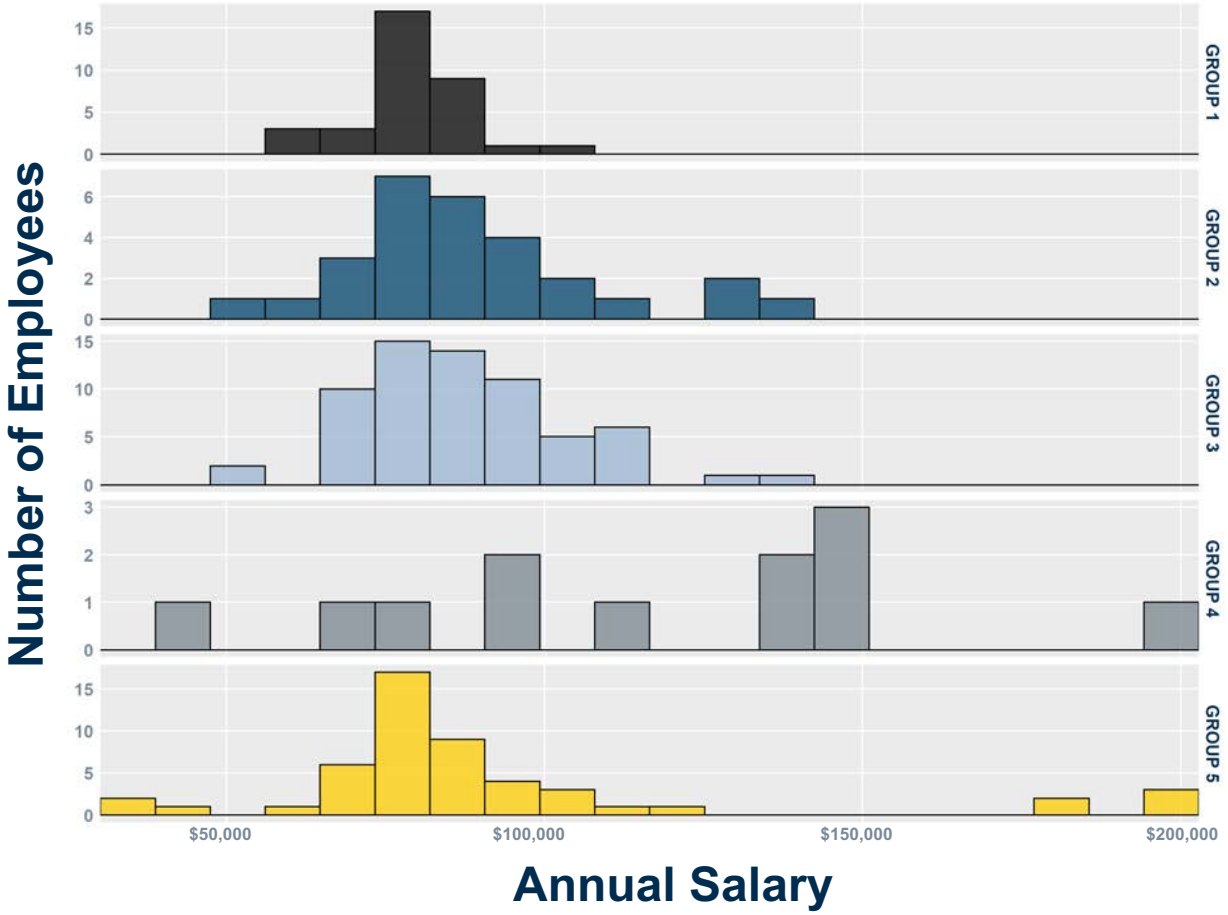


Figure 8. Distributions of annual salary data for clinical directors for each of the five cost-of-living groups. Data are from clinical directors working at organizations that underwent a BHCOE evaluation in 2021 and through July of 2022. Annual salaries were self-reported by organizations during the BHCOE accreditation process.

SECTION II: TURNOVER

Employee turnover, the rate at which employees leave a company, is a widespread source of concern for employers and the recipients of ABA services. Some turnover is expected for every position. For example, employees may leave because they move geographically, or they may leave to pursue higher education. However, voluntary employee turnover may also indicate job dissatisfaction.

New in this report is data on employee tenure. Employees in certain roles within ABA are expected to eventually leave their position within the company. For example, many technicians likely do not remain technicians for the duration of their career as they move to other positions and pursue higher education. Thus, we report average tenure of employees across positions to help organizations benchmark their ability to retain talent.

Employee turnover is a concern because it hinders effective and efficient delivery of therapeutic services. When employees leave, novice employees take the place of colleagues who may have had much more experience and had already built strong rapport with patients. When employees leave, they also take with them the critical knowledge they may have gained over time about the organization and their patients. Further, turnover is associated with lower morale as employees may have to manage an increased workload and it may discourage individuals from applying to open positions. Lastly, losing employees places financial strain on the organization as they spend non-billable resources on recruitment, onboarding, and training of new employees. The disruption in continuity of services can increase patients and their parents/guardian's stress as they work with new employees to catch-up and it may hinder patients' progress.

Several trends emerged with this dataset compared to the BHCOE 2021 report. First, turnover for all three positions decreased in 2021 compared to 2020. Second, the percentage of turnover across all three positions increased steadily from 2016 through 2020 but reduced in 2021. Third, turnover for direct care staff in 2021 remained higher than turnover in 2019, indicating that an increasing trend in turnover may still remain for that position. This makes sense as many organizations pivoted to telehealth and parent training models during COVID-19 which may have reduced job opportunities or the number of available working hours for direct care staff.

COVID-related turnover was not reported as having occurred during the 2021 year. This suggests that the effects of COVID-19 on turnover in the field occurred at the beginning of the pandemic and have largely dissipated by the second year. Alternatively, organizations may have categorized their turnover differently in 2021 as compared to 2020, even though the lingering effects of COVID-19 remained as organizations return to normal operations. We also observed a notable decrease in involuntary turnover for all three positions, and we observed a similarly notable decrease in disciplinary terminations for supervisors and direct care staff. This may reflect the lower unemployment rate in the United States, which could make organizations less likely to terminate employees. Thus, employees who do leave are likely to do so for other reasons.

Overall Turnover Percentages: Direct Care Staff

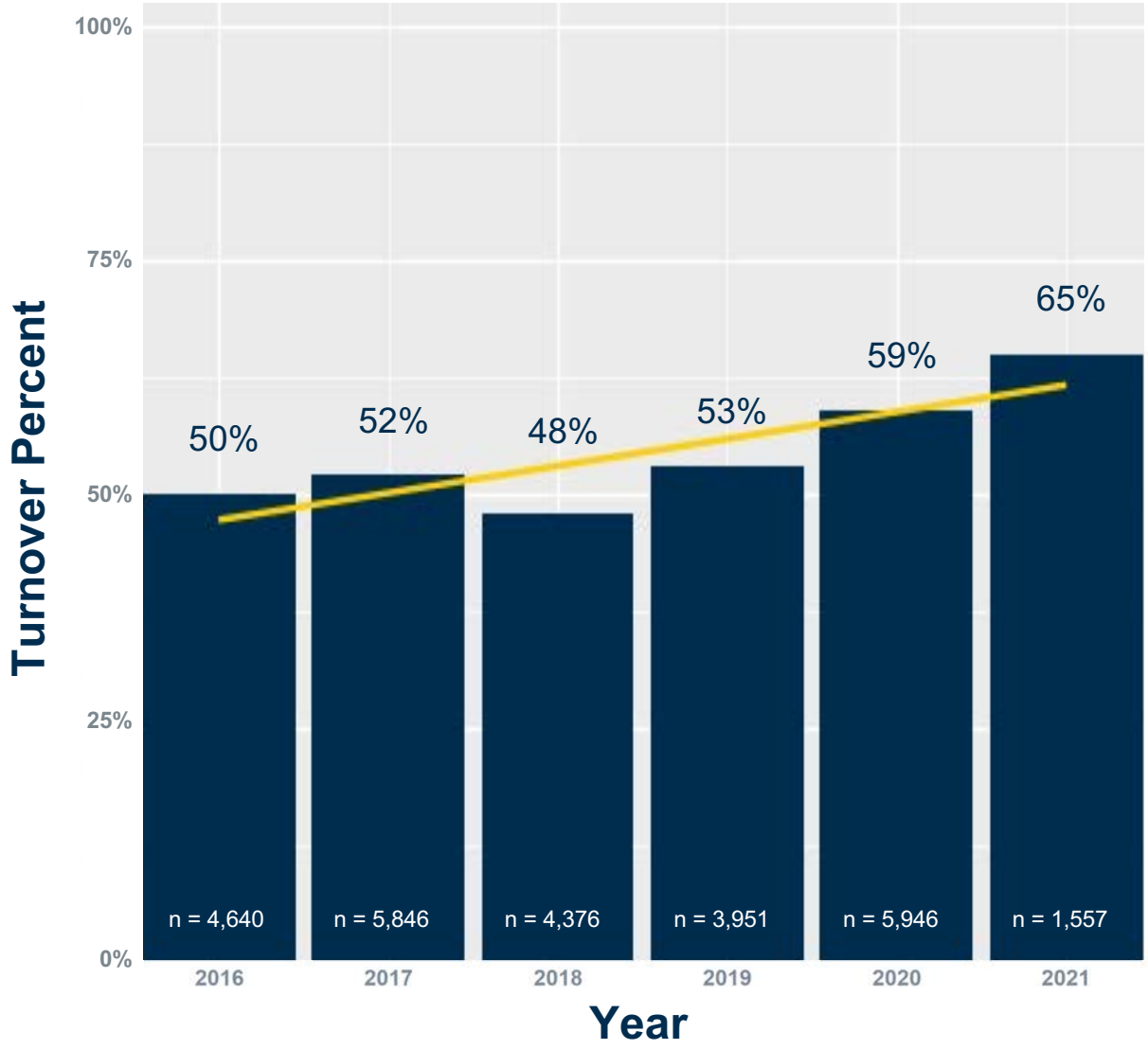


Figure 9. Total annual turnover rate from 2016 to 2021 for direct care staff. Turnover was calculated by totaling the number of separations throughout each year across all organizations, then dividing by the sum of the average number of employees per month. All turnover data was self-reported by organizations participating in the BHCOE accreditation process. Most organizations collect these data regularly through an HR platform. Smaller organizations collect these data manually. Each *n* above indicates the sample size of employees in this position reported for each year of data.

Turnover Percentages by Type: Direct Care Staff

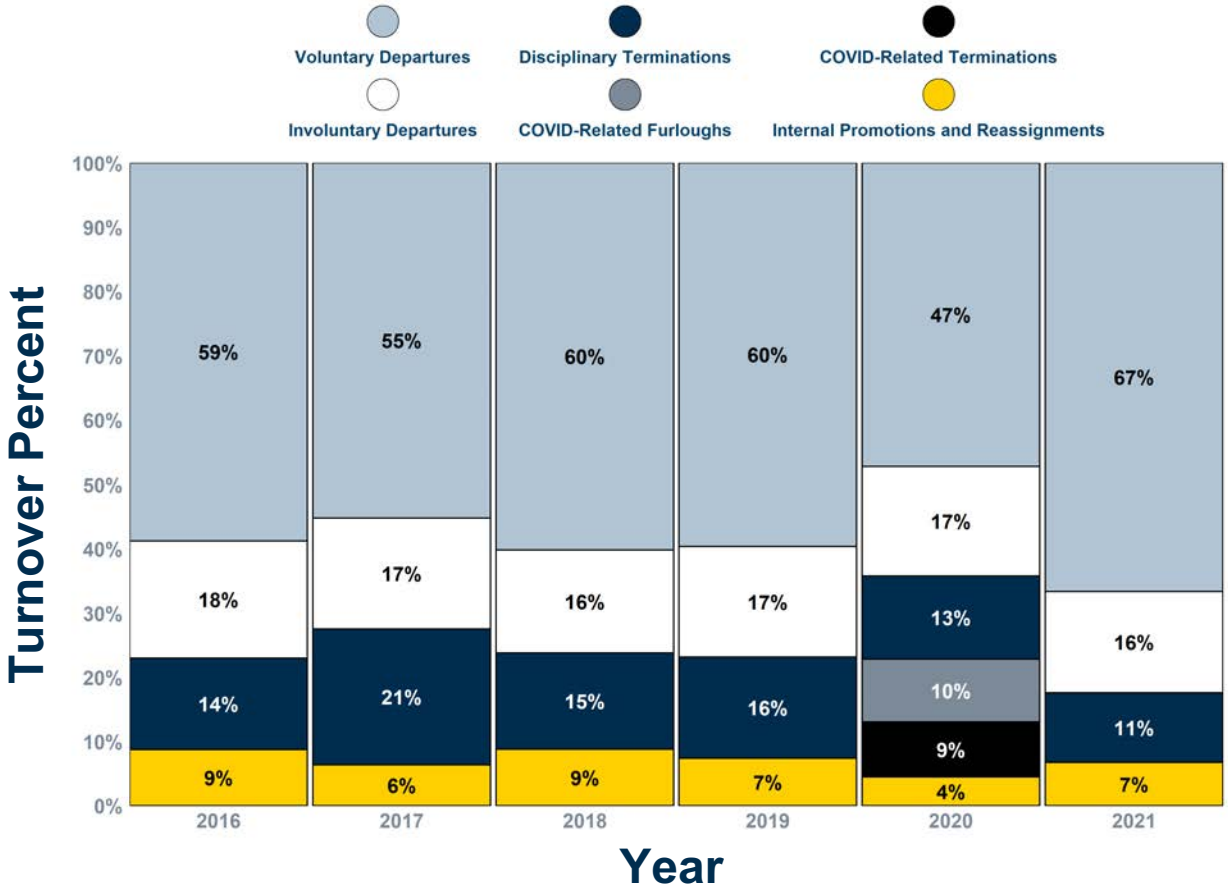


Figure 10. The proportion of total turnover each year accounted for by each turnover category for direct care staff. All turnover data was self-reported by organizations participating in the BHCOE accreditation process. All organizations were provided with the same definitions of each turnover category. Percentages were calculated by summing the total number of reported staff turnover in that category for all organizations, dividing by the total number of staff who left their company, and multiplying by 100.

Overall Turnover Percentages: Supervisory Staff

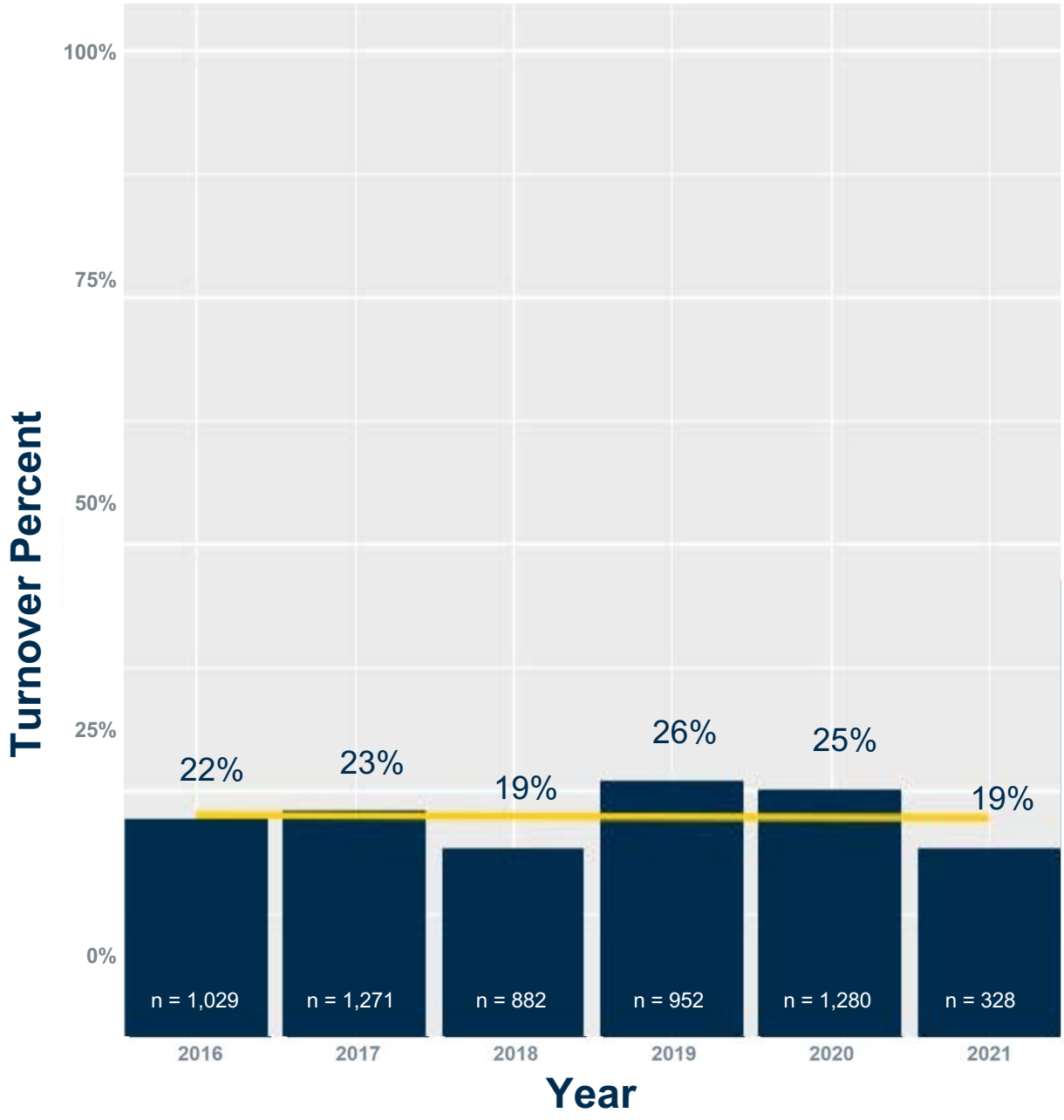


Figure 11. Total annual turnover rate in our data from 2016 to 2021 for supervisory staff. Turnover was calculated by totaling number of separations throughout each year across all organizations, then dividing by the sum of the average number of employees per month. All turnover data was self-reported by organizations participating in the BHCOE accreditation process. Most organizations collect these data regularly through an HR platform. Smaller organizations collect these data manually. Each *n* above indicates the sample size of employees in this position reported for each year of data.

Turnover Percentages by Type: Supervisory Staff

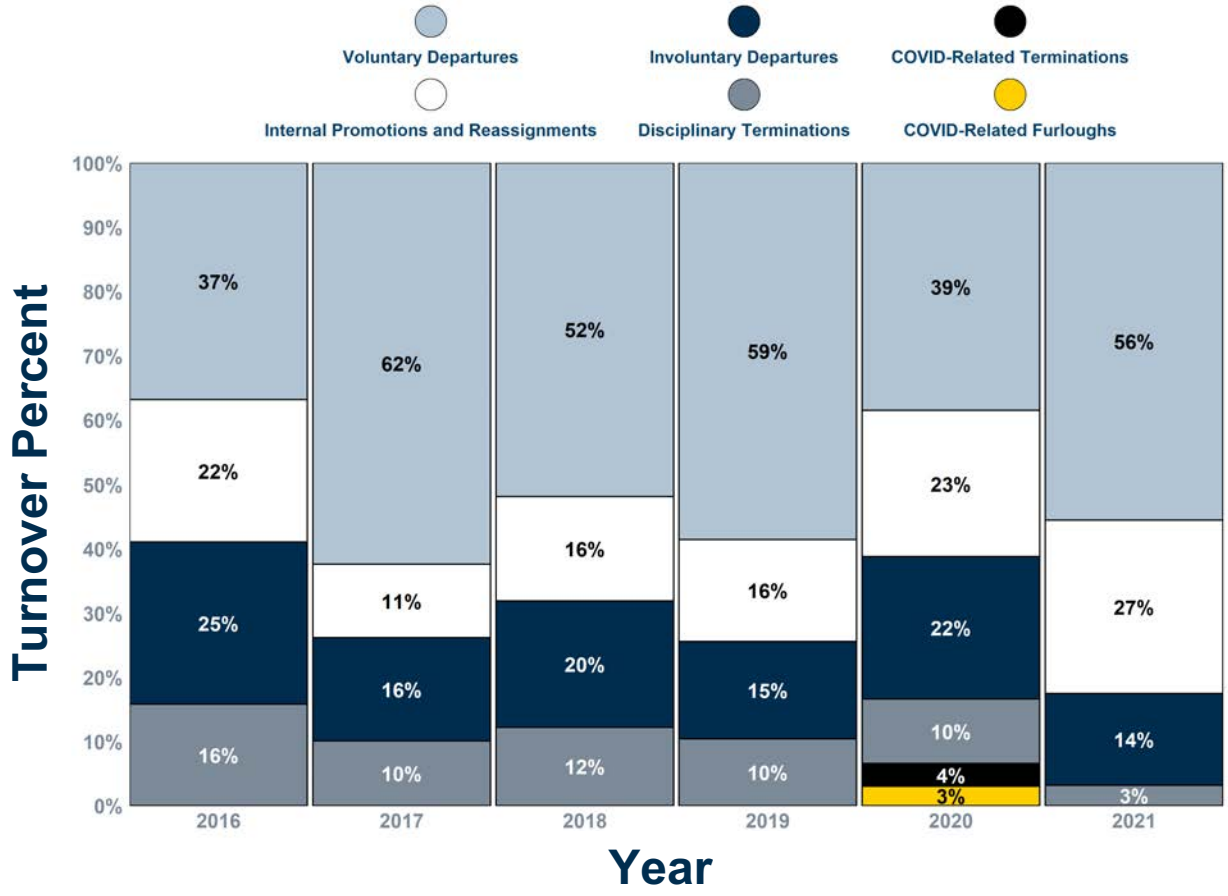


Figure 12. The proportion of total turnover each year accounted for by each turnover category for supervisory staff. All turnover data was self-reported by organizations participating in the BHCOE accreditation process. All organizations were provided with the same definitions of each turnover category. Percentages were calculated by summing the total number of reported staff turnover in that category for all organizations, dividing by the total number of staff who left their company, and multiplying by 100.

Overall Turnover Percentages: Clinical Directors

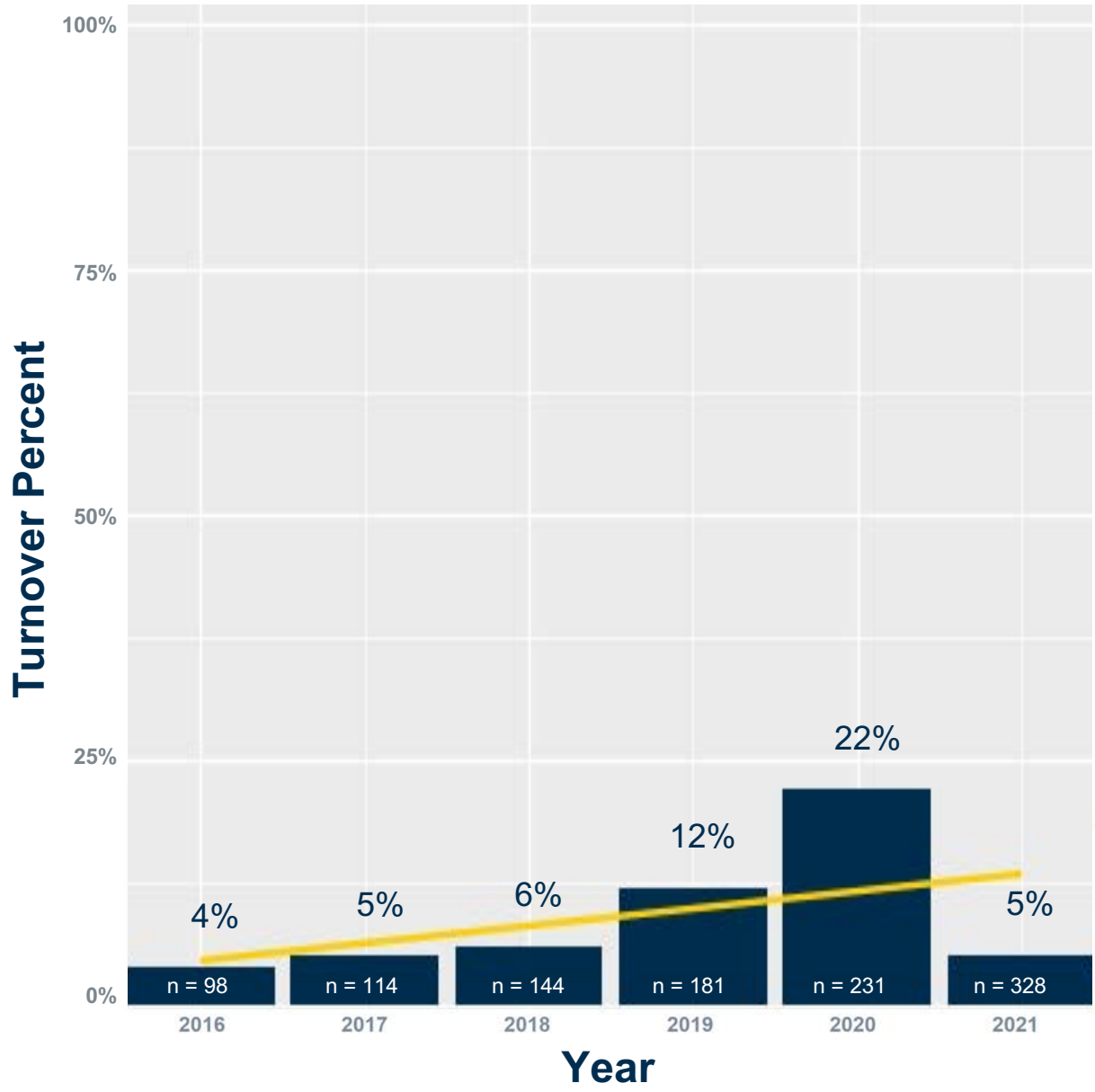


Figure 13. Total annual turnover rate in our data from 2016 to 2021 for clinical directors. Turnover was calculated by totaling number of separations throughout each year across all organizations, then dividing by the sum of the average number of employees per month. All turnover data was self-reported by organizations participating in the BHCOE accreditation process. Most organizations collect these data regularly through an HR platform. Smaller organizations collect these data manually. Each *n* above indicates the sample size of employees in this position reported for each year of data.

Turnover Percentages by Type: Clinical Directors

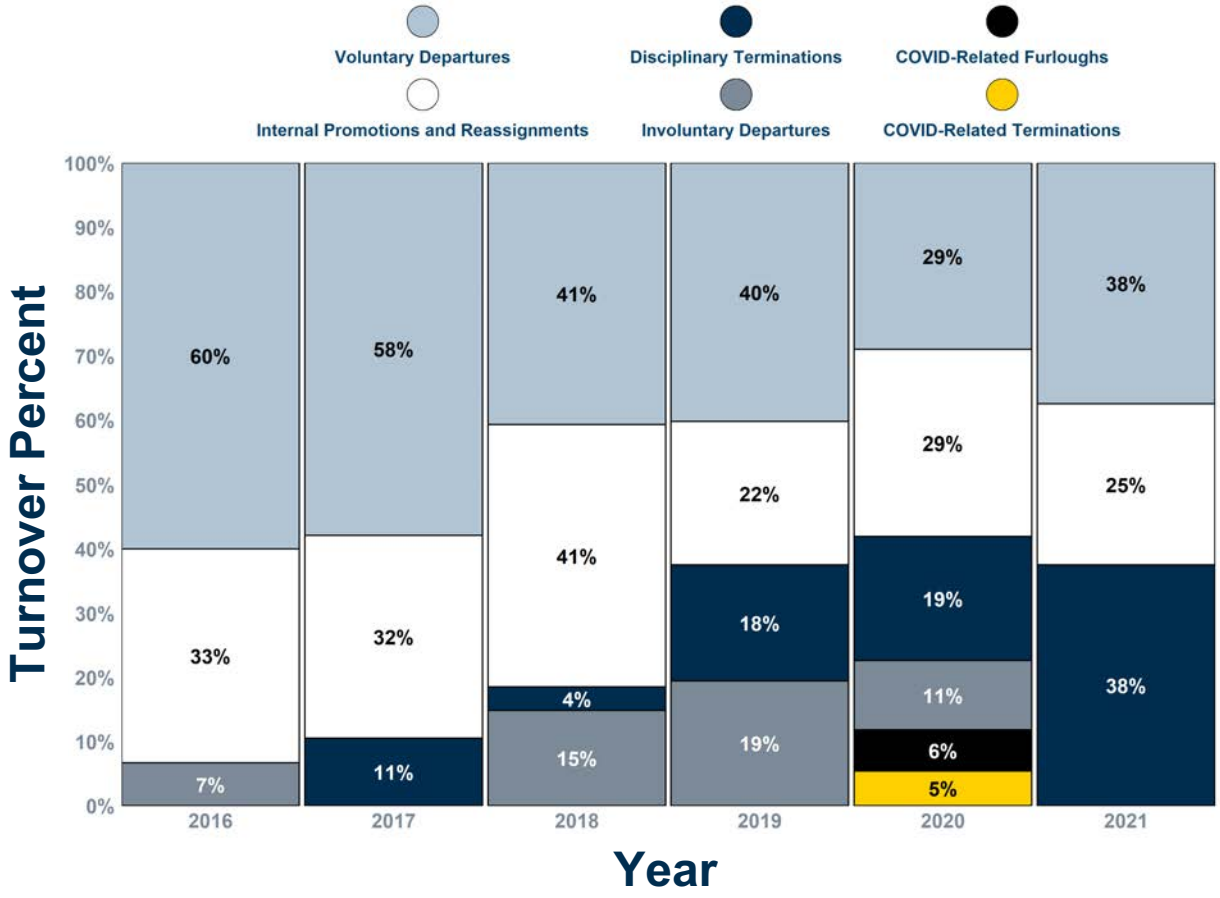


Figure 14. The proportion of total turnover each year accounted for by each turnover category for clinical directors. All turnover data was self-reported by organizations participating in the BHCOE accreditation process. All organizations were provided with the same definitions of each turnover category. Percentages were calculated by summing the total number of reported staff turnover in that category for all organizations, dividing by the total number of staff who left their company, and multiplying by 100.

Employee Tenure at Organizations

Employment statistics suggest that fewer individuals will remain with an organization for the duration of their careers. Further, as entry-level employees (e.g., technicians) accumulate more experience, education, and training, they are likely to move to different roles within the company. That is, most employees are likely to turnover at some point in time across all organizations. Thus, a different way to think about employee retention is through how long employees tend to stay at a company: employee tenure.

For the first time in this report, we include data about how long individuals have been at their respective organizations. The data below were calculated by subtracting an employee's date of hire from the date we received their information, then dividing by 365 to get the number of years the employees in our database had been with the organization.

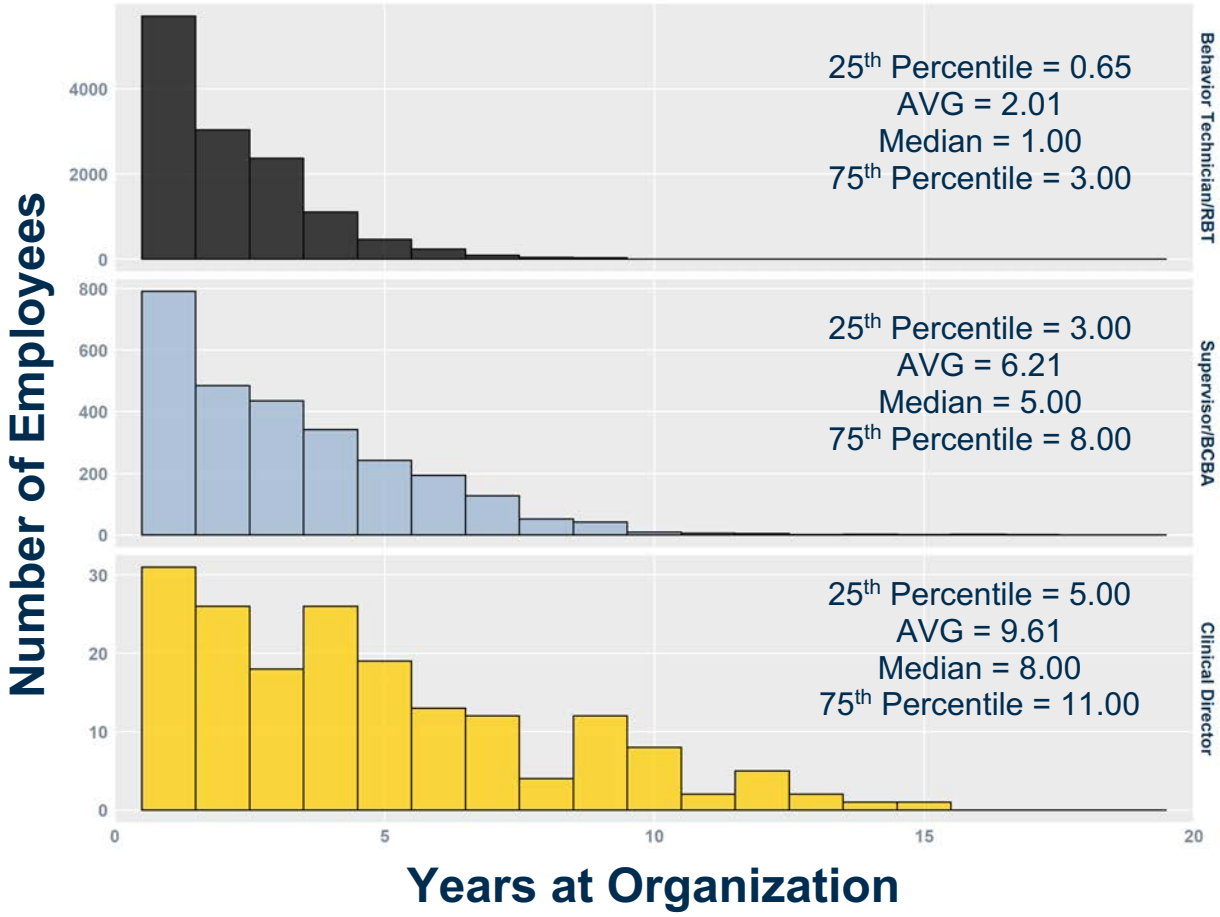


Figure 15. This figure displays distribution of years that all staff have been with their respective organization. Tenure was calculated based on the time between an employee's date of hire and the date BHCOE received the employee's information.

CITING THIS DOCUMENT

This document should be referenced as follows:

Behavioral Health Center of Excellence (2022). 2022 ABA Compensation & Turnover Report, Los Angeles, CA: Author.

ABOUT BHCOE

Behavioral Health Center of Excellence® (BHCOE®) is an organization within the autism field that is dedicated to leading evidence-based quality improvement of equitable Applied Behavior Analysis (ABA) through its creation of performance-based standards for providers and learning and improvement resources for all stakeholders and as the only international accreditor for the field. To learn more about BHCOE, visit bhcoe.org.

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Our community is stronger together. Our doors are always open for like-minded organizations who value ethical services, efficient business practices, standardized peer review, and a community-based approach to behavioral services. If your organization may be a good fit, don't hesitate to let us know.

To begin the evaluation process, submit an application and our clinical team will be in touch to schedule an informational session.

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